

Clinical Findings of Herpes Zoster

Prodrome of Herpes Zoster

Pain and paresthesia in the involved dermatome often precede the rash by several days, ranging from mild itching, tingling, or burning to severe, deep, boring, or lancinating pain. The pain may be constant or intermittent and is frequently associated with tenderness and hyperesthesia in the affected dermatome.

Pre-eruptive pain can mimic conditions such as pleurisy, myocardial infarction, duodenal ulcer, cholecystitis, biliary or renal colic, appendicitis, prolapsed intervertebral disk, or early glaucoma, leading to potential misdiagnosis and inappropriate interventions.

Prodromal pain is rare in immunocompetent individuals under 30 years of age but occurs in most patients over 60. Some patients experience acute segmental neuralgia without developing a skin rash—a condition termed **zoster sine herpete**.

Rash of Herpes Zoster

The hallmark of herpes zoster is its unilateral distribution, typically confined to the skin area innervated by a single sensory ganglion. The rash most commonly affects the dermatomes supplied by the **trigeminal nerve** (especially the ophthalmic division) and the thoracic region (T3 to L2). The thoracic dermatomes account for over half of all cases, and lesions rarely extend beyond the elbows or knees.

Although individual lesions of herpes zoster and varicella are histologically identical, those in herpes zoster evolve more slowly and appear as **closely grouped vesicles on an erythematous base**, in contrast to the scattered, discrete vesicles seen in varicella. This pattern reflects **intraneural spread** of

varicella-zoster virus (VZV) in herpes zoster, as opposed to **hematogenous spread** in varicella.

Lesions begin as **erythematous macules and papules**, often appearing at sites where superficial sensory nerve branches emerge (e.g., posterior and lateral branches of spinal nerves). Vesicles develop within **12 to 24 hours**, progress to **pustules by day 3**, and then **dry and crust over within 7 to 10 days**. Crusts typically remain for **2 to 3 weeks**.

In immunocompetent individuals, new lesions appear for **1 to 4 days**, occasionally up to 7 days. The rash is most severe and prolonged in older adults and mildest and shortest in children.

The incidence and severity of fever are lower in herpes zoster compared to natural varicella.